



Document Management System



Sultanate of Oman
Ministry of Health
The Royal Hospital
Department of Quality and Patient Safety

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Title: Patient Identification & Verification

Definition

It is a method of identifying and labeling all patients/ newborns from admission until discharge from the hospital.

A process of correctly matching a patient to appropriately intended intervention and communicating information about the patients identity

accurately that will help in improving the reliability of the patient's identification process. This process will help eliminate errors and enhance patient care.

Purpose

Providing guidance to all the Royal Hospital's staffs on how to identify patients accurately, correctly and continuously at each point of patient care.

Scope

This policy applies to all health care workers providing care and services to the patients within the Royal Hospital and the centers.

Definitions/Abbreviations:

- **ID band:** Identification band
- **MRN:** Medical Record Number
- **Three V's:** Verbal, Visual and Verification

- **Visualization:** the act or process of looking to the identification details e.g. the visualization of the patient's full four (4) names with tribe,

MRN Number, and patient photo from the system by 2 verifiers.

- **Verification:** the act of confirming and verifying the patient identification details with the patient himself or relatives and against any

request form or documents presented by the patient.

Patient Identification Protocol Sections

Do the Followings

Do not do the Followings

Creation of Identity Band

Bed Side Head Label/ Patient Identification Tag

Applying the Identity Band

When to apply the Identity Band

Patients who do not Wear the Identity Band

Identifying Allergic Patients and Patients with Risk of Fall

Use of Additional Alerts Bands

Procedures Requiring Accurate Identification of Patients

Identifying Patients Undergoing Imaging Procedures

Identifying Patients in the Operating Theatre, Day Care Unit and Endoscopy

Unknown Patients

Infected patients

Deceased Patients

Notifications of Deaths

Missing Identity Bands

Removing the Identity Bands

Reporting Patient Identification Errors

Monitoring Compliance

- a. It is the responsibility of all health care provider to ensure the correct identification band before carrying out any procedures or treatment.
- b. Identify the patient accurately and correctly by using the 3 Vs; verbalization, visualization, and verification
- c. Check the correctness of the identity details with the concerned patient, parents or attendant
- d. All the patients must be called by their full four (4) names and the tribe, and MRN must be checked against the admission or appointment sheet/slip presented by the patient Civil ID Number or Passport Number
- e. Make sure that you always check the details of the patients even if you think that you know them well
- f. Always double-check the identity details on the completed request, order, or prescription form, especially if another member of the healthcare team has completed the form
- g. Attach the printed sticker to the collected samples immediately at the bedside and before you leave the patient room.
- h. Make sure that you always have the correct patient for whatever intervention you plan to carry out by using the 3 Vs
- i. Extra precautions must be taken to alert the staff when there are two or more patients in the same unit/ward having similar names by splitting them in different cubicles.
- j. For the newborn, you must verify the baby with his/her mother ; via applying 2 ID bands with “son of (mother full name)” in baby hand & leg.

2. **Do Not Do the Followings:**

- a. Do Not proceed for any procedure if the patient has no identity band
- b. Do Not rely on patients to identify themselves correctly (depend on patient condition).
- c. Do Not read the patients’ details to patients and allow them to passively agree with you
- d. Do Not walk around with the labels or stickers of different patients stuck to your uniform and lab coat
- e. Do Not write over the old identity band
- f. Do Not take blood from any patient without checking the patient’s details against a fully completed request or order form
- g. Do Not label a sample bottle before you take blood

filling out request forms for several patients at the same time

- i. Do Not print out more patients' labels or stickers than are required at any given time
- j. Do Not open more than one file at a time in the system for documenting the progress notes, to avoid documentation errors in wrong patients' files.

3. Creation of the Identity Band

Identity band/bracelet is to be used in the Royal Hospital for adult and pediatric patients. All written details in the printed patient stickers that

will be attached in the identity band must be only in Black.

The information to be recorded in the white printed sticker and attached in the identity band:

Royal Hospital

MRN

First, second and third name

Family name

Date of Birth

Nationality, Age, Sex, Address

The layout of information on the identity printed sticker:

Royal Hospital

Medical Record Number(MRN)

First, Second, Third and Family name

Date of Birth

Nationality , Age, Sex

Address

4. Precaution for newborn identity band

- a. For all new born babies within the neonatal, paediatric or postnatal wards, 3 identity bands must be used,

b. Before removing the baby from the mother's bedside to the nursery for pediatric procedures, the nurse should check

that the name bands are in place.

c. New Born babies who are still in the neonatal wards, must be visited by their mothers for breast feeding purpose.

If she is unable due to her health condition such as post lower (uterine) segment Caesarean section (LSCS) delivery,

then she will express and send the breast milk to the nursery with the Medical Orderly under the responsibility of the assigned nurse.

Ensure the milk is labelled with the mother's sticker at the bedside while doing milk express.

d. Check that 2 name bands are in place during the daily examination of the newborn; and are replaced if absent, after being checked by 2 Registered Nurses and before re-application in the presence of the mother

or father.

e. If a baby found without name bands, he/she must be identified before re-applying new name bands to the unidentified baby

in the presence of the mother or father.

f. If the baby is transferred to the post-natal ward he/she will retain these identification bracelets until discharge.

g. If a baby is found with both labels missing all other babies in the unit should be checked for adequate ID; the reason for this

should be explained to the parents. If more than one baby is found without labels genetic testing would need to be considered

unless other distinguishing marks allow for positive identification. In such circumstances an incident form should completed

via Al Shifa System.

h. If the baby needs to be taken into a different room, foreexample for a blood sample to be taken, the identificationbands

should be checked with the Mother prior to the babybeing moved and when the baby is returned.

i. When a nurse is involved in feeding a baby, her chair must be placed next to the cot to reduce the risk of replacing the

baby in the wrong cot.

In each bedside area (Adult & Pediatrics) there should be a head label that indicates the name of the admitted patient. The Information

on the bed-head label should include the patient's first, second, third and family name, MRN, patient's consultant name, age and date of birth (for pediatrics only).

6. Applying the Identity Band

- a. Prior applying the ID band to the patient's wrist, 3 Vs must be used and patient must be asked to speak out or state the details of the identity band and to confirm correctness.
- b. In case of emergency or operative situations where clothing is removed, identity band should be attached to the patient's skin using see-through plastic adhesive tape. Some emergency conditions may prevent this, such as burns, so attaching the identity band to clothing may be an appropriate alternative.
- c. The identity band should be fixed on tight enough to avoid it sliding off, but loose enough to prevent constricting circulation. When the patient is very oedematous; two identity bands can be joined together to go around a selected limb (wrist or ankle).
- d. For patients with Arterio-venous shunts or fistula, such as are used in renal dialysis, the identity band should not be fixed to the arm with the shunt.
- e. For all newborn babies within the neonatal or pediatric unit, 3 identity bands must be used, one should be placed in the baby's cot or incubator and the other 2 bands attached in the baby's wrist and ankle or both ankles. All baby's ID bands must be checked in the delivery room against the mother's ID band to ensure accurateness. The presence and accuracy of these must be checked by the registered nurse at the beginning of each episode of care.
- f. All staff applying the patient's identity band must document the creation and placement of it in the patient's health records (Kardex) from admission point; and to verify the patient prior beginning of the shift & ensure it is documented daily & shift wise.

7. When to Apply the Identity Band

- a. The patient's identity band must be applied on admission, or once the patient has entered a department for treatment.
- b. It should be placed by the concerned staff, who will retain responsibility and accountability even when they delegate the task to someone else.

This includes (but not limited):

- Patients in the Emergency Department who have been through triage and are receiving treatment
- Elective in-patients
- Patients in the Day Care Unit
- Patients in the Endoscopy Unit
- Patients in OT or Procedure Room
- All newborns after birth

- a. If the patient does not fit into the above categories (e.g. attendees of out-patient clinics for investigations) and
- b. you are concerned about the safety of the patient, please apply the ID band and remove it when the patient leaves the department.

8. Patients who Do not Wear Identity Band

clearly document the patient's reasons of not wearing the ID band in the patient's health record. Ensuring that patient verification **MUST** be implemented always & before conducting any care to the patient.

b. Patients may be reluctant or be unable to wear the ID band, they need to be well explained & convinced of

the reason of applying ID band, such as those who:

- Refuse to wear the band: Placing the ID band on the ankle instead of the wrist.
- The band causes skin irritation

9. Identifying Allergic Patients and Patients with Risk of Fall

The staff has to label the patient's sticker with red in color see-through plastic adhesive tape (for allergies) and yellow in color

see-through plastic adhesive tape over the patient sticker (for risk of fall)

10. Use of Additional Alert Bands

The use of personal alert bands (Out of allergies and risk of fall) such as those worn by diabetics, epileptics, asthmatics,

and others are allowed, in addition to the hospital ID band.

11. Procedures Requiring Accurate Identification of Patients

a. The accurate identification of any patient must be checked prior to undertaking any procedure and in particular (but not limited):

- Admission, discharge procedure
- Medication administration process
- Handling discharge medication by pharmacist
- X-rays and imaging procedures
- Surgical interventions and any invasive procedures
- Blood transfusion
- Collecting of patient bodily fluid samples
- Blood collection samples
- Transport/transfer of the patient
- Confirmation of death
- consent taken
- Attending appointments in clinics

details by the patient against the fixed identity band and the clinical records.

- b. Always make sure that you don't state the patients' name, and then ask them to confirm or deny by a yes/no reply.
- c. If the patient is unable to tell you their name (e.g. pre-verbal children, dysphasia, expressive disability or mental capacity issues, refer to the ID band and, if possible verify the information by asking family, relatives or another member of clinical staff who is familiar with the patient.
- d. When family member or relatives are not available, two clinical staff will undertake the positive identification and both will sign for the check-in whatever documentation is associated with the process requiring the identification check.

12. Identifying Patients Undergoing Imaging Procedures

- a. It is the ultimate responsibility of the Technician operating the imaging device to ensure that the correct patient is being examined according to the request that has been made. The exposure must not be performed until the patient's identification has been verified.
- b. If the patient details in the request form against the patient's ID band are incomplete or not accurate, further verification must be obtained before an exposure is performed.
- c. Furthermore, the technician **who is calling the ward** has to

- verify him/her self during the call; by stating his/her full name, designation, working department & reason of call.

- He/she has to verify first the staff who answered the call; by asking staff name, designation, & ward name.

- then, to verify about the patient; by asking the staff the patient's full name, IP number, ward name, & patient condition (current condition, infectious state, transfer method, availability of cannula- if required)

- a. The operating technician must correctly **identify the patient prior to performing any exposure**, by:

- Identify the patient accurately and correctly by using the 3 Vs; verbalization, visualization, and verification

- Asking the patient to state their full (4) name.

- Checking the details against the identity band.

- Checking these details against those given on the request form. If the details match, proceed with the exposure. If there is more than one patient on the radiology information system with the same name; double-check the identity against the requested information on the patients file.

- a. If any inpatient arrives for radiographic or radiological examination without the ID band, the patient must be returned to the ward and an incident report should be initiated.

13. Identifying Patients in the Operating Theatre, Day Care Unit and Endoscopy Unit

- a. In the Operating Theatre and Endoscopy unit, the responsible staff in control room/ reception must check the identity of the patient using the theatre checklist and identity band, on line with the approved theatre protocol.
- b. If it is necessary to remove the patient's identity band, prior to or during the operation or the procedure for any reasons, another ID band should be attached again to the patient.

14. Unknown Patients

- a. For unknown or unconscious patients (such as trauma patients), identification will be made by the Medical Record staff in the registration or admission office.
- b. Unknown patients should be registered and given Hospital MRN. with XXXX. Once the patient is identified, the XXXX must be changed and then updated in all records in the online system. The patient's ID band of XXXX must be removed and another new ID band of the approved identity must be applied.
- c. For un-known babies, identification will be made by the medical record staff at the time of admission with XXXX name and MRN number. All XXXX babies must be placed in the nursery with their identification band at all times.
- d. XXXX babies can be given an identified name chosen by the ward nurse or the unit nurse. This is to maintain the dignity and confidentiality of the unknown baby.
- e. The Identified name should be written on the baby's identification card that placed on the head side of the cot/incubator.

15. Infected patients

- a. Any infected patients, such as Covid, should apply ID band as applied for normal clean / non infected patients; ensur following the correct infection control standards/ precautions.

16. Deceased Patients

- a. All deceased patients must be properly identified with two identification bands; one on the wrist and another one on the ankle. As well as another two identifications on the top and botom of the mortuary cover.
- b. In the event of the patient's name not being known, e.g. following a road traffic accident, where the identity is unknown, the identification band must state **UNKNOWN MALE/FEMALE**.
- c. If a limb(s) is missing then attach one label to an available limb and the other to the patient's skin using transparent tape.
- d. **Deseased Infected patients such as Covid;**
 - Ø to apply two identification bands; one on the wrist and another one on the ankle. As well as another two identifications on the top and botom of the **mortuary double cover**.
 - Ø The body will be transported by the municipality ambulance, according to the **Supreme Court Standards**.
 - Ø The last office will be done by municipality, only two members from the first grade family will be allowed to bury the body and they must be wearing full PPE.

17. Notification of Death

The death notification copy Must be given to the family member, and the second copy of death notification Must be taped securely to the outside sheet or body tag (for mortuary verification purpose) and the other copy of the death notification form Must be placed in the health record of the deceased.

18. Missing Identity Bands

records.

If a patient's identity band is found to be missing, accurate patient identification must be checked immediately and replaced on the patient's appropriate limb.

19. Removing the Identify Band

- a. The identity band must remain on the patient during the entire admission and it should only be removed as part of the formal discharge process.
- b. The identity band may be removed once all discharge processes such as drug handover and discharge instruction has been given to the patient, relative or attendant at the point when no further care of patient identification is required.
- c. There should be a genuine reason for the patient identification band to be removed prior to discharge, e.g. its location interferes with the performance of a procedure
- d. on that limb.
- e. The staff who removes the patient's identity band (for any reason) is fully responsible to replace it back following appropriate confirmation of the patient's identity.

20. Patient Identification Errors

Any staff discover patient identification error, must report it immediately to the department or unit in charge.

An incident report must be initiated in online adverse event registration module, in accordance with the Incident reporting and management policy and procedure;

Examples may include:

- a. Wrong information on the identity band
- b. No identity band on the patient
- c. Misidentification of documentation within the health records
- d. Misidentification of X-rays
- e. Misidentification of investigation reports
- f. Misidentification of appointments
- g. Wrong procedure for wrong patient
- h. Medication errors
- i. Blood transfusion errors

21. Monitoring Compliance

The Quality Management Department is responsible for:

- a. Reviewing all the incidents related to patient misidentification every six-months.
- b. Auditing the staff compliance with the patient identification protocol on a monthly basis.

- d. **Route Cause Analysis should be done** for all misidentification incidents directly by the responsible team & department.

Review / Revision History

Revision History			
Revision	Description of change	Author	Revision Date
1	Initial release	Department of Quality and Patient Safety	2016
	Second revision	Department of Quality and Patient Safety	2019
2	Third revision	Nursing Department Department of Quality and Patient Safety	2022

References

Title of book/ journal/ articles/ Website	Author	Year of publication
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Myles Text book for Midwives Churchill Livingstone: London, 14 th edition	Fraser & Cooper	2003

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2014

Written By: Quality & Patient Safety
Checked By: Nursing Team
Authorized by: SUAD SALIM MOHD AL ISMAILI

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